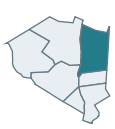
Lyme Disease Burden & Community Action



Mid-Hudson · location only

Prepared 2026-11-05 · Public-data planning brief · Version 1.12

Meeting snapshot · Public evidence and proposed actions for local review

Reported cases · 2024	County rate per 100,000 · 2024	State rate per 100,000 · 2024
1,060	355.5	109.8

Discussion priority: Care without insurance and medical bills

Question for the meeting

Could existing services meet reported uninsured-care needs, or would an additional funded response be needed?

Existing county context: the Dutchess 2025–2030 plan describes access disparities and referral approaches. Discuss how support could complement existing work. [7]

Mid-Hudson counties and state comparison

Area	Reported cases · 2024	Rate per 100,000 · 2024
Dutchess	1,060	355.5
Orange	964	237
Putnam	294	299.2
Rockland	461	135.3
Sullivan	385	482.6
Ulster	841	461.2
Westchester	1,125	113.2
New York State	21,632	109.8

Same NYSDOH 2024 reporting year/source. Cases describe residents, not necessarily exposure location; reported rates are not personal risk estimates.

Study-based economic illustration

USD 2.15 million · 2016 dollars

Reported cases × Hook study societal mean. This illustrates scale, not measured county spending or predicted savings.

Sources [1, 5]; calculation and methods: page 4.

Suggested meeting flow

1. Confirm the local evidence. 2. Discuss one support priority. 3. Check existing services and agree a next step.

Public-data planning brief. Resource context and follow-up: page 2; local planning and research: page 3; references and methods: page 4.

Resources & practical next steps

Healthcare resources and population context

Indicator	Value	Period
Primary-care physicians	233	2023
Hospitals	2	2023
Hospital beds	404	2023
Uninsured residents under age 65	4.7%	2022
Poverty (population with poverty status determined)	8%	2023
Language other than English at home · age 5+	16.2%	ACS 2020–2024

General population/resource context, not Lyme-specific need, appointment availability or interpreter demand. Selected insurance/poverty fields supply no margin of error.

Reported Lyme rates by year · per 100,000 residents

2022	2023	2024
394.6	358.9	355.5

Pre-2022 rates excluded because surveillance definitions changed. Different sources do not establish causes; see methods on page 4.

Existing local resources to check

Tick Task Force · Tick-borne disease prevention and education [10]

Mobile Health Unit · Community outreach and access [11]

Directory snapshot: confirm current activity, eligibility and capacity. Mobile outreach is not verified Lyme treatment; no partnership is implied.

1. Confirm the need with residents

Uninsured care, unaffordable bills and difficulty reaching financial assistance

No county-specific patient-reported findings are established in this brief.

2. Review an existing-service response

Review health-center sliding-fee access, coverage enrollment help and hospital financial assistance with patients; identify staffing, appointment capacity, costs and a possible funding source before proposing new services.

Who could help: Patients, health centers, hospital financial-assistance staff, enrollment assisters, county planners and potential funders

Actions to discuss · Public health departments

- Share tick-prevention information through local events and trusted community channels.
- Review county surveillance alongside service availability; ask residents which barriers need attention.
- Bring residents and potential partners together to choose one support priority and agree how to follow up.

3. Agree responsibility and follow-up

Check services, appointment capacity, staff time, funding and who would take responsibility before proposing a pilot. Define consent and privacy arrangements before any collection.

Choose a common period, such as 30 days. Record the starting situation, support received, missing answers and needs still unmet.

For this priority: Care appointments reached; enrollment assistance completed; financial-assistance decisions; remaining bills and unmet needs over a common period

Proposed actions, not adopted services or demonstrated savings. Professional feedback: source clarity, assumptions, usefulness and missing local factors.

Email feedback: pathwayai.feedback@gmail.com

Local planning & research context

What this means for local planning

1. Prevention outreach

Dutchess's 2024 reported rate was 355.5 per 100,000, above the state rate of 109.8. Where could trusted local channels help share prevention information? [2]

2. Care access

4.7% of residents under age 65 were uninsured in 2022. Could care navigators help residents ask about sliding fees and coverage options? This is general population context, not Lyme-specific need or available appointments. [4]

3. A local action to review

Could existing services meet reported uninsured-care needs, or would an additional funded response be needed? Confirm needs with residents and check existing services before selecting a response.

Planning questions for local review; population indicators do not establish Lyme-specific need or available services.

Fit with the Dutchess 2025–2030 health plan

Local priorities and existing pathways [7]

• The plan describes geographic and disability-related disparities in care access. (pp. 5)

• Among 830 residents participating in informal event voting, insurance coverage and access to care was a leading topic. This was not a representative survey or a Lyme sample. (pp. 6–7)

• The plan describes social-needs referrals through Hudson Valley Care Coalition and Unite Us; its mobile screening pathway concerns blood pressure, not Lyme. (pp. 9–11)

Could reviewed Lyme visit summaries and support questions complement existing care-navigation and referral work? PathwayAI proposal for discussion; no county endorsement, service commitment or referral-platform integration. Verify current services with partners.

Selected CHIP priorities concern nutrition security, chronic-disease prevention and substance-misuse/overdose prevention; these are not an adopted Lyme program.

What published research adds

External study populations provide context, not county patient findings.

Hook et al. (2022) [5]

901 patient-cost participants; 613 societal-cost participants · Recruitment 2014–2016; 2016 dollars

Medical, nonmedical and productivity costs inform the existing illustration.

Population and limits: Selected reported cases in four high-incidence states; excluded non-English speakers and nonacute disease.

Adrion et al. (2015) [13]

52,795 Lyme patients; 263,975 matched controls · Claims 2006–2010; 12-month follow-up

Documents healthcare spending and utilization after Lyme diagnosis.

Population and limits: Commercially insured ages 0–64; not uninsured patients or a full household-cost estimate.

Johnson et al. (2014) [14]

3,090 selected survey respondents · Survey 2013; persistent symptoms after treatment

Documents reported work disruption, expenses and quality-of-life impacts.

Population and limits: Self-selected, self-reported persistent-illness sample; not all Lyme cases or a county disability rate.

What we still need to know

• County-specific unmet care, household spending and caregiving needs have not been established in this brief.

• Long-term Lyme-attributable disability and costs beyond the study follow-up are not estimated.

• Current service eligibility, interpreter access and appointment capacity need local verification.

Missing information is not zero; gaps do not establish an absence of need or services.

Hook is the sole cost basis. Do not add study estimates or infer local disability rates. Local patient findings require a defined collection and review process.

References & methods

Original study basis

1,060 reported cases × USD 2,032 study mean = USD 2,153,920 (2016 dollars).

Hook et al. (published 2022): 613 participants in societal analysis; reported cases recruited in four high-incidence states during 2014–2016. Societal: medical, nonmedical and productivity costs; components are already included.

Applying this study mean to reported county cases is an assumption; the result is not measured local spending. No adjustment for underreporting or local case mix. No causal savings or disability probabilities are inferred.

Hook excluded non-English speakers and nonacute disease. The study does not represent every local household or continuing lifetime costs.

Published cost components · 2016 dollars per participant

Medical: USD 1,333; other expenses and productivity combined: USD 699; total societal mean: USD 2,032.

The combined amount is a derived residual, not a separately reported mean. Caregiving is not separately valued here. Components are already included in the total; do not add them again.

Source register

1. Reported cases, county of residence · 2024

2. Reported rates per 100,000 · 2024

3. CDC surveillance interpretation; historical county file in app · 2019–2022 county file; separate from NYSDOH 2024

4. Healthcare resources, insurance and poverty context · 2022 insurance fields; 2023 healthcare and poverty fields; AHRF 2024–2025 release

5. Hook et al. Economic Burden of Reported Lyme Disease in High-Incidence Areas, United States, 2014–2016. Emerging Infectious Diseases. 2022. · 2016 dollars; recruitment 2014–2016

6. County location graphic: bundled boundaries · Bundled boundary file; underlying vintage not independently verified

7. Dutchess Community Health Improvement Plan · 2025–2030; printed pp. 5–11

8. Shared Mid-Hudson regional assessment (Dutchess and Orange), Figure 147, page 219 · 2022–2023; regional assessment published 2025

9. Language spoken at home, age 5+ · ACS 2020–2024

10. Tick Task Force · Existing directory; current status requires verification

11. Mobile Health Unit · Existing directory; current status requires verification

12. Community Health Improvement Plan · Existing directory; current status requires verification

13. Adrion et al. Health Care Costs, Utilization and Patterns of Care following Lyme Disease. PLOS ONE. 2015;10:e0116767. · Claims 2006–2010; 12-month follow-up

14. Johnson et al. Severity of chronic Lyme disease compared to other chronic conditions: a quality of life survey. PeerJ. 2014;2:e322. · Survey 2013; persistent symptoms after treatment

Verification and interpretation

Existing source snapshots; newer official releases have not been verified for this brief. Official rates are reproduced; exact rate-denominator vintage is not independently verified. Pre-2022 rates are excluded because surveillance definitions changed.

Census language context is an existing QuickFacts snapshot, not a fresh pull or a measure of English proficiency. HRSA field years are not substituted with newer Census periods. Initiative links require current local verification; no listing does not mean no services exist.

Location map uses the app’s bundled county boundaries for orientation only; underlying boundary vintage is not independently verified. It is not an official county map or a disease-risk map.

Public evidence and proposed planning actions only. No session stories, personal records or synthetic patient cards are included.

Discussion brief, not a county demand assessment, benefits decision, funded service or adopted county plan. No county or organizational endorsement is implied.